

DISCHARGE SUMMARY

ILLUSTRATIVE EXAMPLE — fictional patient, fictional details, not a real medical record

Example Care Facility

Patient & Admission Details

Patient name	A. Kumar (fictional)
Patient reference	EX-P001 (fictional)
Age / Sex	34 years / Male
Facility	Example Care Facility
Ward / Department	Sample ward — General Medicine
Admission date	10 September 2026
Discharge date	12 September 2026
Length of stay	2 days
Consultant	Dr. R. Sharma (fictional)

Diagnosis

Acute viral gastroenteritis with mild dehydration.

Presenting Complaints

- Loose stools (5–6 episodes/day) for 2 days prior to admission
- Associated vomiting and low-grade fever
- Generalised weakness and reduced oral intake

Course in Hospital / Treatment Summary

Patient was admitted with the above complaints. On examination, mild dehydration was noted with stable vital signs. Routine blood investigations and stool examination were sent, which were unremarkable apart from mild electrolyte imbalance. Patient was managed with IV fluid rehydration, antiemetics, and antipyretics. Symptoms improved steadily over the course of admission, and oral intake was gradually resumed. Patient remained hemodynamically stable throughout the stay and was ambulatory prior to discharge.

Condition at Discharge

Stable, afebrile, tolerating oral intake well, and ambulatory without assistance. Vitals within normal limits at the time of discharge.

Discharge Medications

- Tab. Ondansetron 4 mg — SOS for nausea, up to 3 days
- Oral Rehydration Solution — as needed for 5 days
- Tab. Paracetamol 500 mg — SOS for fever, up to 3 days
- Probiotic capsule — once daily for 5 days

Diet & Activity Advice

- Light, easily digestible diet (khichdi, curd rice, bananas) for the next 3–4 days
- Avoid oily, spicy, and street food until fully recovered
- Adequate oral fluid intake through the day
- Gradual return to normal activity as tolerated

Follow-up Plan

Review in General Medicine OPD after 5 days, or earlier if symptoms recur or worsen (persistent vomiting, high fever, blood in stools, or signs of dehydration). Report to the nearest emergency department if red-flag symptoms develop.

This discharge summary is a fictional, illustrative example created for demonstration purposes only. The patient, clinician, diagnosis, and clinical details are invented and do not correspond to any real person or medical event. It must not be used for actual patient care, insurance, legal, or billing purposes.